nearly 4.5 per cent of all generic drugs circulated in the domestic market were substandard. Poor quality medicines from India have been making global headlines. In 2022, 65 child deaths were linked to contaminated cough syrup made in India by a company called Marion Biotech. In August, the WHO issued a global alert about an Indian-made cold syrup called Cold Out, which is sold in Iraq. "I would not prescribe unbranded medicine to my family. There are many of highly dubious quality and others that don't even work. How can I be forced to do so for patients?" says a doctor from a leading government hospital in Delhi, who does not wish to be named.

Doctors are also outraged at what they perceive is a shift in decision-making power from physicians to pharmacists. By simply writing the generic name on a prescription, they are allowing pharmacists to decide which company's unbranded generic drug the patient should take. "Treatment is not as straightforward as that—we need to take into account side-effects, medical history, allergies etc. before prescribing a drug. How can the NMC decide that the best person to determine which medicine a patient should take is a chemist?" asks Dr Kumar. In the event that an unbranded medicine fails to have an effect, or worse, has serious side-effects, doctors are worried about their reputation as well as violence from affected patients and their family. "Violence against doctors is a serious issue. I have had experience of patients, spouses or children being aggressive," says Dr Ashutosh Sen, a general physician from Kolkata. "If I face the liability of my prescription, then I must also be allowed to prescribe a drug of my choice. Suppose the chemist prescribes a medicine from a brand that is not good quality and anything happens to a patient...my head will be on the chopping block," he adds.

Proponents of generic drugs point to the alleged role of big pharma in pressuring doctors to prescribe branded medicines. "This has never been proven and if the public starts to regard doctors as money-makers alone, there will be severe consequences. As doctors, we are under oath to save lives and that is our only mandate," says Dr Kumar.



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WHAT SHOULD BE DONE

In the August 21 meeting with Mandaviya, one of the proposals was to reduce the prices of branded generic medicines. "Reduction of tax can help reduce prices of branded generics. There should be rules to ensure quality, distribution and chemist regulation. We feel that the approach should be more flexible, allowing the doctor's judgement to prevail," says Dr Girish Mathur, president, Association of Physicians in India.

Finally, the government has begun to take steps to improve the quality of medical manufacturing. Of India's 10,500 medicine manufacturing units, around 8,500 are in the MSME category. Only 2,000 units in this category have WHO GMP (good manufacturing practice) certification. Quality control for these come under Schedule M of the Drugs Rules, 1945, which govern production units and their maintenance, personnel, control and safety testing, storage and transport of material, written procedures and traceability. Last month, drug regulators inspected 162 units and 14 public testing labs and found lapses such as lack of process and analytical validations, absence of quality failure investigation and internal quality review, as well as faulty design of manufacturing and testing areas. Based on these, GMP recommendations in the current Schedule M have been revised to reflect globally-accepted standards. Pharma companies with an annual turnover of over Rs 250 crore will have to adopt GMP recommendations in the next six months. Units with a turnover under that have been given an additional six months to comply.

The government is also trying to ensure quality control of around 1,800 unbranded generics sold at its 9,600 Jan Aushadhi stores. "We do third-party and anonymous testing for our batches and only source from WHO GMP-certified firms after inspecting their facilities," says Jan Aushadhi CEO Ravi Dadhich. "The batches are tested for permissible impurities, hardness, dissolution levels and effectiveness." The chain has only four warehouses that supply all its stores, thereby ensuring each store keeps approved medicine. "In the last nine years, we have saved Rs 20,000 crore for Indians, with Rs 7,000 crore saved last year alone," says Dadhich. He insists Jan Aushadhi can supply quality medicine at the most affordable price (at times, almost 90 per cent less than branded generics) with virtually no risk of adulteration or tampering.

Ultimately, say experts, the goal should be to make healthcare more affordable without compromising on quality of drugs. For this, reduction in taxation, more economical manufacturing infrastructure and stringent monitoring of all processes will yield better results than to compel doctors to relinquish crucial decision-making power and prescribe only a certain type of medicine. ■